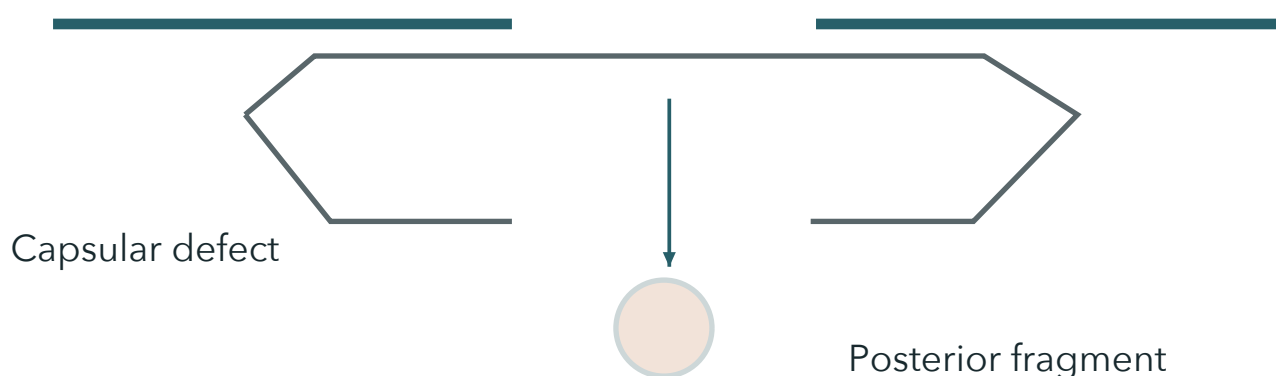


Retained lens material and postoperative inflammation

Part 2 | Posteriorly retained fragments need a posterior plan

Do not chase material blindly through a capsular defect.

Anterior working plane



Original compartment section: a posterior fragment lies beyond the safe anterior working plane.

Stabilize the eye and assess vitreous, residual material, pressure and inflammation. Coordinate vitreoretinal review when nuclear material has dropped posteriorly. Timing of definitive removal depends on the fragment, clinical course and available expertise; an anterior maneuver that adds traction can worsen the retinal risk. Document the event and arrange an explicit follow-up plan.

Lens-related pressure is not one syndrome

Capsular integrity and released material distinguish mechanisms.

Mechanism	Key substrate
Phacomorphic angle closure	Swollen lens crowds the anterior segment
Phacolytic obstruction	Leaked proteins from a mature lens
Lens-particle obstruction	Freed lens material after capsule disruption
Phacoantigenic inflammation	Immune reaction to exposed lens proteins

Control pressure and inflammation while addressing the source. Gonioscopy, capsule status and the operative or trauma history help distinguish the pathways. A high IOP does not automatically establish pupil block, and an open angle does not exclude a lens-related cause.

TASS is a patient and a systems problem

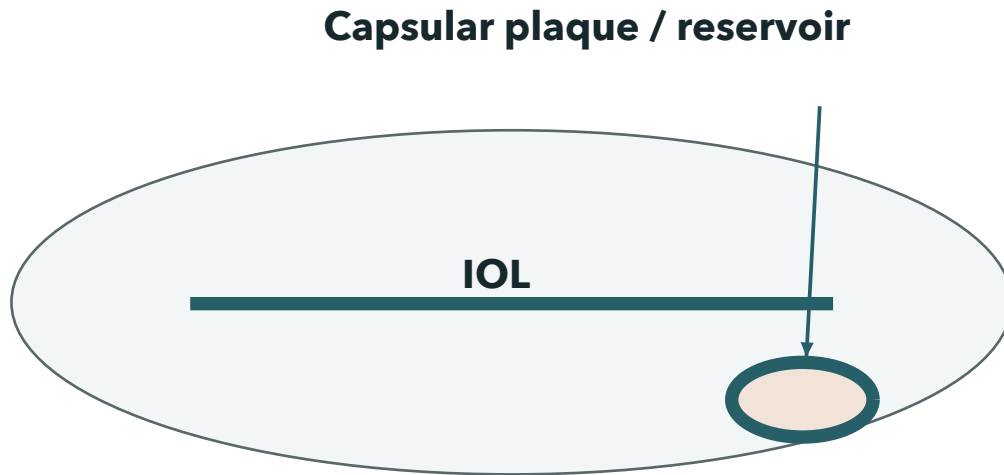
Treat the eye while investigating a shared toxic exposure.

Clinical work	Systems work
Exclude infection when uncertain	Preserve traceability and implicated batch details
Control sterile inflammation	Review solutions and medication preparation
Monitor cornea and IOP	Review cleaning and instrument processing
Reassess unexpected progression	Prevent repeat exposure while investigating

A cluster after one operating list raises concern about common processes. Sterilization success does not prove the absence of toxic residues. A responding eye does not remove the need for service-level investigation; appropriate reporting and a documented corrective action protect later patients.

Recurrent delayed inflammation can hide a capsular reservoir

A negative routine aqueous culture is not definitive exclusion.



Target capsule, not only free aqueous.

Original section: organisms can persist within the capsular bag around a plaque.

C. acnes may cause steroid-responsive recurrent inflammation months after surgery. Targeted capsular material and prolonged anaerobic culture can improve recovery; PCR and culture are complementary, with either sometimes positive when the other is negative. Coordinate sampling and laboratory methods before adding more immunosuppression to a presumed autoimmune flare.

Definitive treatment must address the reservoir

Acute-endophthalmitis trial rules do not answer every chronic infection.

Question	Why it matters
Where is the organism sequestered?	Drug exposure may not eliminate a capsular focus
How often does inflammation recur?	Relapse suggests persistent source
What tissue and implant can be retained?	Procedure must balance eradication and support
Is vitreous involved?	Changes sampling and operative planning

Management may require intraocular antimicrobial treatment with selected capsular removal, vitrectomy or implant removal according to the case. Small retrospective series do not establish one mandatory operation for every patient. Distinguish eradicating the source from temporarily suppressing the inflammatory response.